



I/DD, BUSINESS ACUMEN, AND MANAGED CARE: WHAT'S NEXT?

SELN Annual Meeting
October 2, 2018
Alexandria, VA
Laura Vegas





Business Acumen for Disability Organizations Grant



2



- 3 years
 - October 1, 2016 – September 30, 2019
- Funded by the Administration for Community Living
- Goal/Vision:
 - Build the capacity of disability community organizations to contract with integrated care and other health sector entities
 - Improve the ability of disability networks to act as active stakeholders in the development and implementation of integrated systems within their state

Grant Objectives



9

- Develop baseline knowledge of current community-based organizations
- Provide broad-based training and technical assistance for disability networks to build their capacity
- Convene and provide targeted technical assistance utilizing a learning collaborative model
- Engage integrated care organizations, managed care plans, and other health care entities regarding the needs of consumers and the roles of CBOs

Outcomes



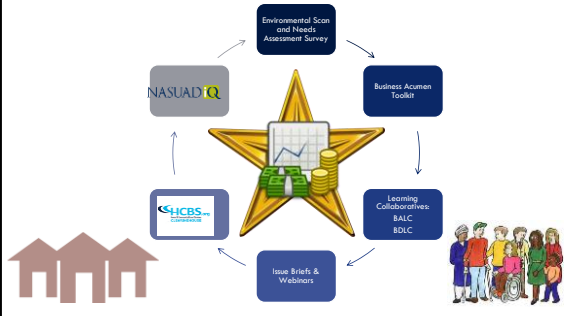
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- Increased knowledge of current CBO successes, challenges, needs, and promising practices
- Increased technical assistance and business acumen resources to support CBOs
- Increase in learning collaborative participants' business capacity to engage with integrated care networks
- The improvement of health care entities' awareness about the role CBOs can play in the health care system

Sustainability via Business Acumen



5



HCBS Business Acumen Resource Center Website



<http://www.hcbsbusinessacumen.org/>

- Environmental Scan & Needs Assessment Survey
- Webinars
- Toolkit
- Learning Collaborative
- Partner Organizations



DISABILITY NETWORK BUSINESS STRATEGIES: A Roadmap to Financial and Programmatic Sustainability for Community-Based Organizations



7

■ Prepare

- Organization Vision
- Environmental Scan
- SWOT Analysis
- Champion Development

■ Plan

- Analyze
- Prioritize
- Organize
- Manage

■ Execute

- Develop and sustain relationships and partnerships
- Negotiate and contract
- Manage risk

■ Monitor/Evaluate

- Continuous quality improvement
- Compliance - meet contract expectations
- Modify approaches

Webinars



8

- Business Acumen 101
- Business Acumen Toolkit
- Stories from the Field



<http://www.hcbsbusinessacumen.org/webinars.html>

Webinars



9

2017

- Business Acumen 101: Modernizing Your Community Based Business in a Changing Environment
- Managed Care – understanding the changing environment
- Perspectives and Language – Business, Health/Medical, LTSS, Disabilities/Aging
- Are they buying what you're selling - An inside look at what health plans need from community based organizations
- From mission to fruition: Developing your relationships with payers
- Business Acumen: Tools to help you succeed!
- Planning to Plan? Tools to help you better understand your current business environment.
- Information to Action: Planning and Change Management
- Developing Relationships with Potential Payers and Community Partners

2018

- Strategy Development: Use national data to inform your work. How to use the National Core Indicators and National Core Indicators - Aging and Disabilities to develop contracting strategies
- Stories from the Field: Lessons of sharing from the Disability Network Business Acumen Learning Collaborative
- Supporting Community Based Organizations in their Move to Managed Care: One Health Plan's Experience.
- Independent Practice Associations: What Are they, how do they work, and Will they benefit me?
- Translating Data into Better Outcomes: Practical Approaches to Using Data to Facilitate Change
- MLTSS for People with Intellectual and Developmental Disabilities: Strategies for Success
- Stories from the Field Contest Winners

Stories from the Field



10

- **United Disabilities Services (UDS)**, a non-profit organization committed to serving people with disabilities in Pennsylvania.
 - Developed a quality management program, that includes multiple measures, to help build their value proposition.
- **Area Agency on Aging District 7 (AAA7)**, a AAA that serves individuals of all ages and disability types who reside in southern Ohio.
 - Developed a bid to provide case management services, and successfully became a subcontractor for a managed care company in 2014.
- **Community Options Enterprises**, a subsidiary of Community Options, a national non-profit organization.
 - Developed the Daily Plan It, a complete office, conference, and copy center that offers office rental space or virtual tenancy to a variety of businesses. The Daily Plan It which provides an avenue for people with disabilities to work and is a self-sustaining business model with a hybrid payer structure, where customer purchased services and office rentals support the cost of operations.

Two Collaboratives – Two Objectives



11

- **Business Acumen Learning Collaborative**
 - Develop and implement business-related strategies to state-specific challenges to integrating long term services and supports and healthcare services



- **Shared Aim:** Develop Resources to Support CBOs to Identify, Manage, and Mitigate Risks Related to Integrated Care

Two Collaboratives – Two Objectives



12

- **Business Development Learning Collaborative**
 - Evaluate the CBO business environment and develop business strategies to strengthen and sustain community-based organizations that serve people with disabilities



- **Shared Aim:** prepare CBOs to pivot and pro-actively respond to a changing environment to ensure quality services

Insights from the LCs: Lessons Learned



13

- Breadth and Depth of Culture Change
 - ▣ Industry Disruption
 - ▣ Business Acumen Critical for Everyone
- Power of Collaboration – Developing Solutions Together
 - ▣ Significance of Meaningful Stakeholder Engagement
 - ▣ Recognizing “Expertise” Around the Table
- Understanding Our Value
 - ▣ We Did Social Determinants Before They Were Cool
 - ▣ Demonstrating Our Impact

Insights from the LCs – Opportunities & Needs



14

- Focus on Community Education and Buy-In
 - ▣ Just Scratching the Surface
 - ▣ Getting Below the Interested
- Digging Into the Process
 - ▣ Building Out Execute
 - ▣ Practical experiences and tangible tools
- Measuring Our Value – Connecting to Payment
 - ▣ Identifying Outcomes
 - ▣ Quantifying Impact

Prepare: Resources for CBOs



Disability Network Business Strategies: A Roadmap to Financial and Programmatic Sustainability for Community-Based Organizations

Step 1: Prepare

Understand the Business Environment and Your Place Within It
<http://nasuad.org/sites/nasuad/files/BACStep1Prepare.pdf>



Webinars:

- Business Acumen 101: Modernizing Your Community Based Business in a Changing Environment (Webinar)
- Are We Saying the Same Thing? The Language of Long Term Services and Supports and Managed Care (Webinar)
- Are They Buying What You're Selling? An Inside Look at What Health Plans Need from Community Based Organizations (Webinar)
- From Mission to Fruition: Developing your Relationships with Payers (Webinar)

All located at <http://www.hcbsbusinessacumen.org/webinar-archive.html>

Prepare: Resources for CBOs



Develop and Implement a Strategic Business Plan

- Planning to Plan? Tools to Use to Help You Better Understand your Current Business Environment (Webinar)
- Information to Action: Strategic Planning and Change Management (Webinar)
- Expand Your Vision and Grow Your Mission! Driving Culture Change in Aging and Disability CBOs Looking to Work with Health Care Partners
- Leadership and Change Management for Community Based Organizations

Webinars located at <http://www.hcbsbusinessacumen.org/webinar-archive.html>
Other Resources locate at <http://www.hcbsbusinessacumen.org/successful-organizational-change.html>



17

Describe: The Current MLTSS Landscape

Kim Opsahl, ANCOR, Director of State Partnerships & Special Projects

Medicaid Long-Term Services & Supports



Source: RUSHD survey, CMS data

Medicaid Expenditures & LTSS



24% vs 7%

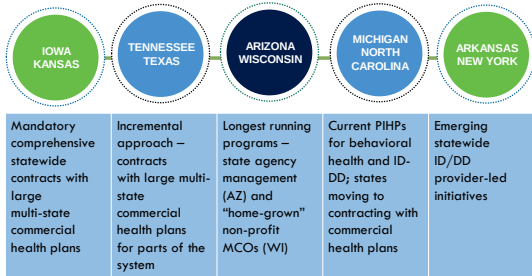
Of All Medicaid
LTSS Spending for
Older Adults and
People with
Physical Disabilities

Of All Medicaid
LTSS Spending for
People with I/DD

State LTSS Approaches



20



Source: HMA Presentation to ANCOR Board of Directors, April 2018

21

Key Issues and Challenges: LTSS for Individuals with I/DD

Lack of Potential Cost Savings



Limited Savings Opportunities

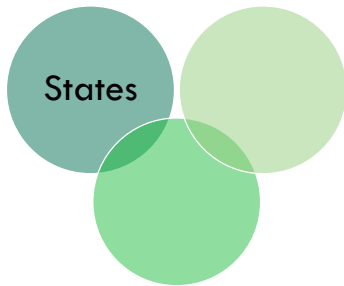
Limited
Funding
Options

Reliance on
Public
Funding

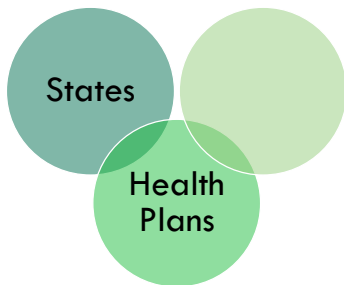
Impact of
Direct
Support

Limited
Opportunity
for
Rebalancing

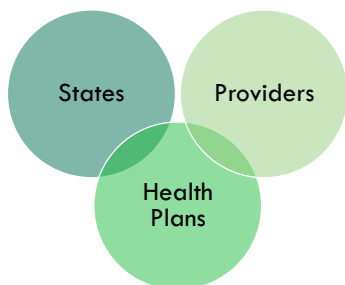
Limited Experience Serving People with I/DD in MLTSS



Limited Experience Serving People with I/DD in MLTSS



Limited Experience Serving People with I/DD in MLTSS



Need for Quality Measures



- Lack of valid and reliable metrics to measure LTSS outcomes
 - Not widely agreed upon
 - Not easily defined
- Evidence-based outcome measurement is evolving
- Concerns regarding a return to a "medical model"

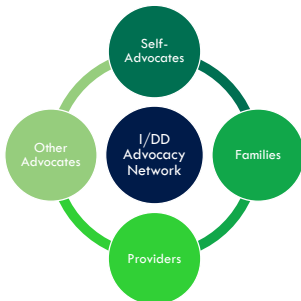
Unique Approach to Case Management & Supports Coordination

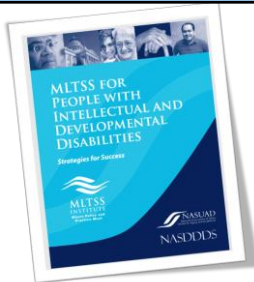


Unique Approach to Case Management & Supports Coordination



Strong Advocacy Networks & Relationships





30

Strategies for Success

Laura Vegas, NASDDDS, Director of Business Acumen

31

Why Do States Implement MLTSS?

State Goals & Expectations



11

- Accountability rests with a single entity
 - ▣ Financial risk for health plan provides opportunity to incentivize/penalize performance
 - ▣ Plans can integrate siloed streams of care (primary/behavioral/long term care) more effectively
- Administrative simplification for state
 - ▣ Eliminates need to contract with and monitor hundreds/thousands of individual providers
 - ▣ Managed care plans take on claims payment, member management, utilization review, etc.

State Goals & Expectations



12

- Budget Predictability
 - ▣ Capitation payments greatly minimize unanticipated spending
 - ▣ Can more accurately project costs (especially with LTSS as enrollment doesn't have as much variation based on economic circumstances)
- Shift services to community settings
 - ▣ Most consumers express preference for community-based services
 - ▣ Health plans have demonstrated effectiveness in diverting and reducing institutional stays

State Goals and Expectations



13

■ Innovation and Quality

- MCOs can deliver services more flexibly than states
- National companies, in particular, can bring best practices from other states/product lines
- Local plans are grounded in their communities
- Demonstrated improvement in quality outcomes (HEDIS) over FFS

■ Consumer becomes the center, not their services

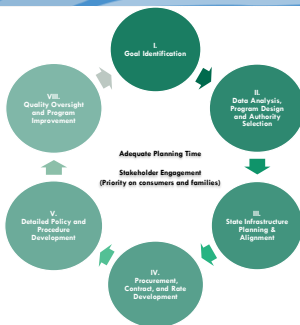
- LTSS interventions can lower acute care costs
- Increased likelihood of 'bending the cost curve'

35

MLTSS Program Lifecycle



36



Adequate Planning Time



37

- Adequate planning time is critical pre-requisite for success
 - CMS recommends two year planning process, minimum
 - Thoughtful planning and design
 - Incorporate stakeholder input
 - Safeguards to insure smooth transition
 - Education about MLTSS for people with I/DD and their families
 - Health plans need to familiarize themselves with LTSS for people with I/DD and their local provider network
 - Provider network may need more time to adjust to health plan requirements

Adequate Planning Time – Examples



38

- **Tennessee** spent more than two years in planning their MLTSS program for people with I/DD, called Employment and Community First CHOICES (ECF)
- **Wisconsin** phased in its FamilyCare program gradually across the state by county

Stakeholder Engagement



39

- Stakeholder engagement cuts across all phases of the lifecycle
- Should be broad-based
 - Advocates
 - Providers
 - Health plans
 - Community-based organizations
 - Parents/families (more to come)
- Two-way street
 - Incorporate stakeholder recommendations into program design
 - Providing TA to self-advocates, families, providers

Stakeholder Engagement



40

- Health plans must have process for stakeholder engagement
- Stakeholder engagement – families/caregivers should get particular attention
 - Meaningful roles in the design and implementation of managed care for people with I/DD and their families
 - States and managed care partners must have strong transparent systems for meaningful stakeholder engagement
 - Opportunities for input and response to stakeholders
 - Adequate time for engagement and review of successes and challenges of implementation
 - Ongoing, not just during design and implementation

Stakeholder Engagement – Examples



41

- **Tennessee's** Medicaid agency and DD agency facilitated stakeholder feedback by holding in-person meetings with key provider and advocacy groups, community meetings around the state, as well as an on-line survey.
- **North Carolina** hosted listening sessions across the state, invited written comments and supplemented open invitation for written feedback with targeted outreach to consumers, their families and providers.
- **Sunflower Health Plan's** I/DD LTSS manager participated in development of Kansas' MLTSS program.

MLTSS Program Lifecycle – Goal Identification



42

- State system goals should also incorporate goals for participants including person centeredness, employment choices, and self-determination
- The I/DD system is typically 'rebalanced' so other goals will be predominant
 - Increased access to preventive and acute services
 - Comprehensive care/service coordination
 - Budget predictability and stability

Goal Identification - Examples



43

- Michigan's program includes specific goals for all participants to pursue competitive employment.
- Tennessee's ECF CHOICES program was designed to make integrated competitive employment and community living the first and preferred option for people with I/DD.

MLTSS Program Lifecycle – Program Design



44

- Data on FFS costs, utilization and provider distribution is critical
 - Set adequate capitation rates
 - Identify network capacity/gaps
- Populations, benefit package, geographic reach all need to be identified
- Decision on managed care authority and/or modifications to HCBS service authority
- Maintain historical I/DD quality infrastructure (ie. NCI)



Kansas continues to administer NCI survey and requires MCOs to use data in their QI activities

MLTSS Program Lifecycle – State Infrastructure



45

- Close collaboration with I/DD operating agency/unit to access experience for design and implementation plans
 - Voc rehab and education play important role as well
- If feasible, transition waiver oversight staff to contract/quality monitoring; specific focus on critical incident reporting and monitoring
- Ensure beneficiary support system staff have training/experience with I/DD participants and services



Tennessee's Medicaid and Vocational Rehab agencies signed an MOU to ensure consistent delivery of employment services

MLTSS Program Lifecycle – Procurement/Rates



46

- RFP should make state expectations and priorities clear
 - Include program requirements specific to I/DD populations
 - Seek demonstrated expertise/philosophy about I/DD populations
- Establish minimum standards for provider ‘credentials’ and other I/DD program-specific elements
- Adequate rates are essential, and should support goals of program
 - Encourage innovation but maintain stability of system

MLTSS Program Lifecycle – Contracts



47

- Specific MCO staff composition and skill set (different from other populations)
- Minimize burden on providers (standardized processes across plans)
- Expectations for addressing informal support network, esp. family members
- Collect and submit data to show progress to goals (ie. competitive employment achieved and maintained)

Contracts - Examples



48

- Tennessee developed ‘preferred contracting standards’ for ECF CHOICES provider network
 - Focused on bringing providers that were in agreement and supported the program’s goals of integrated competitive employment and community integration
- Michigan includes a specific Employment Works Policy in its PIHP contracts
- Kansas specifies integrated employment outcomes that MCOs must achieve and tracks MCO success in meeting them.

MLTSS Program Lifecycle – Policies and Procedures



49

- The devil is in the details
- Contract cannot enumerate the operational aspects of program implementation
- The more information the state can transmit increases likelihood of health plans implementing the program design with fidelity
- Transparent communication is critical
- Educate and train, and educate and train some more

Policies and Procedures - Examples



50

- 3 plans in North Carolina collaborate to deliver web-based PCP training to providers using DirectCourse
- BlueCare Tennessee and Amerigroup Tennessee collaborated on training for providers interested in participating in the ECF CHOICES program

MLTSS Program Lifecycle – Quality Improvement



51

- Important to establish quality measures that will assess success in meeting program goals
- Participant input critical to defining ‘success’
 - Clinical quality secondary to meeting participants goals for their lives
- Ensuring health and welfare a key quality outcome
- Wealth of NCI historical data can be instrumental in comparing experiences under MLTSS



The I/DD agency in Tennessee conducts on-site MCO performance reviews

Key Takeaways



52

- MLTSS programs for people with I/DD hold great promise for expanding employment and improving health status
- Deliberate and thoughtful design, procurement and implementation plans will increase likelihood of success
- Slow and steady wins the race!
- States and MCOs should work collaboratively to support providers and achieve program goals
- Regular and bidirectional engagement with participants and their families will provide important feedback loop
- Measuring quality in meaningful ways will support program success

Questions?



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